

GA Healthcare Training & Consulting

Nurse Aide Training Program (NATP) — Blended Learning Course

SKILLS LAB & CLINICAL LESSON PLANS

7-Part Federal & State Core Curriculum (42 CFR §483.152 / Georgia DCH NATP)

Aligned to: Hartman's Nursing Assistant Care: The Basics, 6th Edition (Fuzy/Hartman Publishing, ISBN 978-1-60425-141-8)

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Program Totals: 40 hrs online (TalentLMS) · 34 hrs in-person lab · 24 hrs clinical · 98 total program hours

Last Updated: August 2026

How to Use This Lesson Plan Packet

This packet contains instructor lesson plans for every in-person skills lab session and clinical day in the blended NATP course. Didactic content is delivered online through TalentLMS before each lab session; lab time is reserved for demonstration, supervised practice, and return demonstration. Each lesson plan is keyed to a Part of the Federal and State Core Curriculum, with Hartman 6e page references, GA DCH/OBRA compliance notes, and required skills checklists. All textbook citations have been verified against the Hartman 6e table of contents and procedure list: Appendix A maps every chapter and learning objective to the curriculum Parts, and Appendix B indexes each Hartman procedure by page for lab demonstration and clinical validation.

Federal rule: a minimum of 16 hours of training in Part I subjects must be completed before any direct contact with a resident (42 CFR §483.152(b)(1)). This program allocates 22 pre-clinical hours to Part I, exceeding the federal minimum. All Part I lesson plans must be completed and documented before Week 4 clinical.

Hour Allocation Summary — In-Person Lab (34 hrs)

Curriculum Part	Topic	Lab Hrs	When
Part I-A	Communication, interpersonal skills, stress management, chain of command	4	Week 1 (pre-clinical)
Part I-B	Infection control	4	Week 1 (pre-clinical)
Part I-C	Safety / emergency / FBAO / body mechanics	4	Week 1 (pre-clinical)
Part I-D	Promoting residents' independence	4	Week 2 (pre-clinical)
Part I-E	Respecting residents' rights	2	Week 2 (pre-clinical)
Part I-F	Legal/ethical behavior and scope of practice	4	Week 2 (pre-clinical)
Part II	Basic nursing skills (vital signs, height/weight, observation, end-of-life)	3	Week 3
Part III	Personal care skills (bathing, grooming, toileting, feeding, skin, transfers)	4	Week 3
Part IV	Mental health and social service needs	1	Week 3
Part V	Care of cognitively impaired residents	1	Week 3
Part VI	Basic restorative services	2	Week 3
Part VII	Residents' rights (behavioral demonstration)	1	Week 3
	TOTAL LAB	34	

Clinical Practice (24 hrs)

Monday–Wednesday, Week 4 | 7:00 a.m.–3:00 p.m. | 8 hours per day at the affiliated long-term care facility, under direct supervision of the RN Instructor of Record. Instructor-to-student ratio maintained per Georgia DCH requirements. Clinical Day lesson plans appear at the end of this packet.

PART I — Pre-Clinical Core (22 Hours)

All six Part I lesson plans must be completed, with return demonstrations documented, prior to any direct resident contact.

Lesson Plan I-A: Communication & Interpersonal Skills, Stress Management, Chain of Command

Curriculum Part	Part I-A
Setting	GAHCTC Skills Laboratory
Allocated Hours	4 hours (pre-clinical)
Textbook Reference	Hartman 6e, Ch. 2, pp. 21–30 (communication, barriers, special-needs communication); Ch. 1, pp. 4–9 (care team, chain of command, professionalism); Ch. 10, pp. 252–257 (conflict, feedback, stress management)
GA NATP / OBRA Note	GA NATP Mod 02 · OBRA pre-clinical subject Federal 16-hr rule
Pre-Clinical Requirement	YES — required before resident contact

LEARNING OBJECTIVES

Upon completion, the student will be able to:

1. Demonstrate the elements of the communication process (sender, message, receiver, feedback).
2. Use verbal and nonverbal techniques appropriate to residents with hearing, vision, and speech impairments.
3. Demonstrate accurate, objective reporting up the chain of command.
4. Distinguish objective observations from subjective statements when reporting and recording.
5. Identify personal stress signals and describe two healthy stress-management strategies.
6. Demonstrate answering a call light and communicating a resident concern to the charge nurse.

MATERIALS & EQUIPMENT

- Call light/pendant system or simulated call bell
- Communication scenario cards (hearing-impaired, vision-impaired, aphasia, angry family member)
- Sample organizational chart of a long-term care facility
- Blank observation/reporting worksheets; whiteboard
- Hartman Workbook Ch. 2 exercises

LESSON SEQUENCE & TIME ALLOCATION

Time	Activity	Method
0:00–0:30	Warm-up & TalentLMS check: review of communication process; barriers to communication discussion	Instructor-led discussion
0:30–1:15	Demonstration: therapeutic verbal/nonverbal communication; introducing yourself, knocking, addressing residents by preferred name	Instructor demo + guided role play
1:15–2:00	Station rotation: communicating with hearing-impaired, vision-impaired, and aphasic residents (scenario cards)	Small-group practice

Time	Activity	Method
2:00–2:45	Chain of command drill: 'who do you report to?' — resident change-of-condition scenarios reported to the charge nurse using objective language	Role play + return demo
2:45–3:30	Stress management: recognizing burnout; box-breathing and boundary-setting practice; group debrief	Guided practice
3:30–4:00	Return demonstrations & documentation; Workbook Ch. 2 review; exit ticket	Evaluation

SKILLS CHECKLIST — RETURN DEMONSTRATION REQUIRED

Each skill is demonstrated by the instructor, practiced under supervision, and validated by return demonstration on the GAHCTC skills checklist. Students must achieve 100% on critical steps before clinical placement.

- Answers call light within scope, identifies self, addresses resident by name
- Reports a change of condition to supervisor using objective, factual language
- Demonstrates communication adaptation for a sensory-impaired resident

EVALUATION & DOCUMENTATION

- Instructor-observed return demonstration on GAHCTC communication checklist (satisfactory/unsatisfactory)
- Chain-of-command scenario graded with rubric; must correctly identify reporting pathway
- Attendance and hours logged on the Part I hour-tracking form

INSTRUCTOR NOTES

Reinforce that the CNA reports to the licensed nurse — never diagnoses, never gives medical advice. Connect to Module 01 (Roles & Responsibilities) content completed online.

Lesson Plan I-B: Infection Control

Curriculum Part	Part I-B
Setting	GAHCTC Skills Laboratory
Allocated Hours	4 hours (pre-clinical)
Textbook Reference	Hartman 6e, Ch. 2, pp. 46–62 (infection prevention and control practices) · Procedures: hand hygiene p. 50; donning/doffing gown p. 51; mask and goggles p. 52; donning gloves p. 53; removing gloves p. 54
GA NATP / OBRA Note	GA NATP Mod 04 · Standard & Transmission-Based Precautions Federal 16-hr rule
Pre-Clinical Requirement	YES — required before resident contact; 100% on hand hygiene & PPE before clinical

LEARNING OBJECTIVES

Upon completion, the student will be able to:

1. Explain the chain of infection and how standard precautions break it.
2. Demonstrate proper hand hygiene (handwashing and alcohol-based rub) per CDC technique.
3. Demonstrate donning and doffing PPE (gown, mask, goggles, gloves) in correct order without self-contamination.
4. Identify when transmission-based precautions (contact, droplet, airborne) apply.
5. Demonstrate correct handling of clean vs. dirty linen and disposal of contaminated materials.

MATERIALS & EQUIPMENT

- Sinks or portable handwashing stations; soap, paper towels, ABHR
- Glo Germ kit with UV light (hand hygiene validation)
- PPE sets: gowns, gloves (multiple sizes), surgical masks, eye protection
- Isolation signage set (contact/droplet/airborne); linen bags, biohazard bags
- Hartman skills video: Hand Hygiene; Donning/Doffing PPE

LESSON SEQUENCE & TIME ALLOCATION

Time	Activity	Method
0:00–0:20	Chain of infection rapid review; why CNAs are the front line of infection prevention	Discussion
0:20–1:00	Hand hygiene: instructor demonstration, Glo Germ practice, individual validation under UV light	Demo + return demo
1:00–2:00	PPE donning/doffing: demonstration, paired practice with peer-checking of sequence, individual validation	Demo + return demo
2:00–2:45	Transmission-based precautions stations: match the sign to the scenario; entering/exiting an isolation room	Station rotation
2:45–3:30	Linen handling, spill response, sharps awareness, and cleaning of shared equipment	Demo + practice
3:30–4:00	Skills checklist completion; remediation for any student not at 100%; documentation	Evaluation

SKILLS CHECKLIST — RETURN DEMONSTRATION REQUIRED

Each skill is demonstrated by the instructor, practiced under supervision, and validated by return demonstration on the GAHCTC skills checklist. Students must achieve 100% on critical steps before clinical placement.

- Hand hygiene — handwashing (minimum 20 seconds, all surfaces, no recontamination)
- Hand hygiene — alcohol-based hand rub
- Donning PPE in correct order (gown → mask → goggles → gloves)
- Doffing PPE in correct order without contamination (gloves → goggles → gown → mask)
- Handling soiled linen away from uniform; correct bagging

EVALUATION & DOCUMENTATION

- 100% competency required on hand hygiene and PPE checklists before clinical — mandatory gate
- UV-light validation photo/log retained in student file
- Hours documented on Part I tracking form

INSTRUCTOR NOTES

Hand hygiene and PPE are among the most heavily tested skills on the Georgia state exam. Repeat validation on Clinical Day 1 before care assignments.

Lesson Plan I-C: Safety, Emergency Procedures, FBAO, and Body Mechanics

Curriculum Part	Part I-C
Setting	GAHCTC Skills Laboratory
Allocated Hours	4 hours (pre-clinical)
Textbook Reference	Hartman 6e, Ch. 2, pp. 31–45 (safety and nonmedical emergencies; medical emergencies); Ch. 7, pp. 195–197 (restraints and restraint-free environment) · Procedures: abdominal thrusts p. 38; shock p. 39; myocardial infarction p. 40; bleeding p. 41; burns p. 41; fainting p. 42; seizures p. 43; vomiting p. 45
GA NATP / OBRA Note	GA NATP Mod 03 · Safety & Emergency Federal 16-hr rule
Pre-Clinical Requirement	YES — required before resident contact

LEARNING OBJECTIVES

Upon completion, the student will be able to:

1. Identify common safety hazards and fall-risk factors in long-term care.
2. Demonstrate principles of good body mechanics when lifting, moving, and repositioning.
3. Demonstrate abdominal thrusts for a conscious adult with foreign-body airway obstruction (FBAO) and describe response for an unconscious victim.
4. Describe the RACE and PASS protocols for fire emergency response.
5. Demonstrate correct response to a witnessed fall, including not moving the resident and reporting.
6. Identify restraint alternatives and the hazards of restraint use.

MATERIALS & EQUIPMENT

- Choking rescue manikin or training vest; adult CPR manikin (awareness only — BLS certified separately)
- Gait belts; hospital bed; wheelchair with locking brakes
- Fire extinguisher trainer (or PASS demonstration model); evacuation map
- Fall scenario cards; incident report forms (facility sample)

LESSON SEQUENCE & TIME ALLOCATION

Time	Activity	Method
0:00–0:30	Safety hazard scavenger hunt: identify staged hazards in the lab room; fall-risk factors discussion	Active learning
0:30–1:15	Body mechanics: wide base, bend at knees, keep load close, raise bed height, no twisting — demonstration and practice moving supplies and repositioning a peer	Demo + practice
1:15–2:15	FBAO: recognizing choking, abdominal thrusts on manikin/vest; response to unconscious victim; when to call for the nurse and activate EMS	Demo + return demo
2:15–3:00	Fire safety: RACE and PASS walk-through; evacuation roles of the CNA; oxygen safety precautions	Demo + drill
3:00–3:40	Witnessed fall scenario: stay with resident, call for help, do not move, observe, report; complete a practice incident report	Simulation

Time	Activity	Method
3:40–4:00	Return demonstrations; checklist documentation; exit questions	Evaluation

SKILLS CHECKLIST — RETURN DEMONSTRATION REQUIRED

Each skill is demonstrated by the instructor, practiced under supervision, and validated by return demonstration on the GAHCTC skills checklist. Students must achieve 100% on critical steps before clinical placement.

- Demonstrates proper body mechanics during a lift/reposition task
- Performs abdominal thrusts (FBAO — conscious adult) on training device
- Verbalizes RACE and PASS correctly in a fire scenario
- Responds correctly to a witnessed resident fall

EVALUATION & DOCUMENTATION

- Return demonstration checklists (satisfactory required on FBAO and body mechanics)
- Scenario-based oral questioning on fire and fall response
- Hours documented on Part I tracking form

INSTRUCTOR NOTES

Clarify that NATP FBAO instruction does not replace AHA BLS certification; students may complete BLS through GAHCTC's AHA courses.

Lesson Plan I-D: Promoting Residents' Independence

Curriculum Part	Part I-D
Setting	GAHCTC Skills Laboratory
Allocated Hours	4 hours (pre-clinical)
Textbook Reference	Hartman 6e, Ch. 3, pp. 65–69 (holistic care, promoting independence and self-care, cultural differences, need for activity, family roles); Ch. 1, pp. 4–9 (nursing assistant role, care team, professionalism); Ch. 9, pp. 235–237 (restorative care, promoting independence)
GA NATP / OBRA Note	GA NATP Mod 01 · Mod 05 · Person-centered care (OBRA 1987)
Pre-Clinical Requirement	YES — required before resident contact

LEARNING OBJECTIVES

Upon completion, the student will be able to:

1. Explain why independence and self-care matter to residents' physical and emotional health.
2. Demonstrate encouraging a resident to perform tasks at their own ability level rather than doing tasks for them.
3. Demonstrate offering choices during care (clothing, timing, sequence).
4. Identify assistive approaches that support independence in ADLs.
5. Describe the consequences of learned helplessness and loss of function.

MATERIALS & EQUIPMENT

- ADL practice supplies: clothing items, grooming kit, meal tray setup
- Scenario cards: resident who is slow but able; resident who refuses help; family member who insists on 'doing everything'
- Timer (to practice patience pacing)

LESSON SEQUENCE & TIME ALLOCATION

Time	Activity	Method
0:00–0:30	Discussion: 'use it or lose it' — function, dignity, and OBRA's mandate for highest practicable well-being	Discussion
0:30–1:30	Paired practice: assisting with dressing while prompting the resident to do each achievable step; instructor coaches cueing language ('You do the buttons, I'll hold the sleeve')	Guided practice
1:30–2:30	Choice-offering drills across ADLs: meals, grooming, activity participation; practicing wait time without taking over	Role play
2:30–3:30	Case rotation: adapting when a resident is fatigued, in pain, or refuses; balancing safety with autonomy; when to report declining function	Small-group cases
3:30–4:00	Return demonstration: 'assist, don't do' scenario; debrief and documentation	Evaluation

SKILLS CHECKLIST — RETURN DEMONSTRATION REQUIRED

Each skill is demonstrated by the instructor, practiced under supervision, and validated by return demonstration on the GAHCTC skills checklist. Students must achieve 100% on critical steps before clinical placement.

- Prompts and cues a resident through a self-care task rather than performing it for them
- Offers at least two genuine choices during a care interaction
- Verbalizes when and how to report a change in a resident's functional ability

EVALUATION & DOCUMENTATION

- Instructor-observed 'assist, don't do' return demonstration with rubric
- Participation in case rotations; reflective exit ticket
- Hours documented on Part I tracking form

INSTRUCTOR NOTES

This lesson sets the person-centered tone carried through Parts III and VI. Connect to restorative care philosophy (Ch. 9).

Lesson Plan I-E: Respecting Residents' Rights

Curriculum Part	Part I-E
Setting	GAHCTC Skills Laboratory
Allocated Hours	2 hours (pre-clinical)
Textbook Reference	Hartman 6e, Ch. 1, pp. 10–20 (legal and ethical behavior, Residents' Rights, legal aspects of the medical record, MDS, incident reports); Ch. 3, pp. 65–68 (holistic care, independence, cultural differences, family roles)
GA NATP / OBRA Note	GA NATP Mod 05 · Residents' Rights (mandatory pre-clinical topic, OBRA 1987)
Pre-Clinical Requirement	YES — required before resident contact

LEARNING OBJECTIVES

Upon completion, the student will be able to:

1. List key rights guaranteed under OBRA 1987, including dignity, privacy, choice, and freedom from abuse and restraint.
2. Demonstrate privacy measures during care (knock, close door and curtain, cover the resident).
3. Explain confidentiality (HIPAA) as applied to hallway, elevator, and social media situations.
4. Describe the CNA's mandatory reporting duty for suspected abuse, neglect, or mistreatment.
5. Identify the role of the long-term care ombudsman.

MATERIALS & EQUIPMENT

- Residents' Rights poster materials (group exercise from course map)
- Privacy demonstration setup: bed, privacy curtain, bath blanket
- HIPAA scenario cards; abuse-reporting decision flowchart

LESSON SEQUENCE & TIME ALLOCATION

Time	Activity	Method
0:00–0:25	OBRA 1987 rights review; group creates Residents' Rights poster for the lab wall	Group activity
0:25–1:00	Privacy in practice: knock-announce-wait, curtain/door, draping during care — demonstration and paired practice	Demo + return demo
1:00–1:35	HIPAA scenarios: what can you say, to whom, where? Social media pitfalls for healthcare workers	Case discussion
1:35–1:50	Abuse/neglect recognition and mandatory reporting pathway; ombudsman role	Instructor-led
1:50–2:00	Checklist documentation and exit questions	Evaluation

SKILLS CHECKLIST — RETURN DEMONSTRATION REQUIRED

Each skill is demonstrated by the instructor, practiced under supervision, and validated by return demonstration on the GAHCTC skills checklist. Students must achieve 100% on critical steps before clinical placement.

- Demonstrates knock, self-introduction, and privacy provision before care
- Maintains draping/covering during a simulated care task

- Verbalizes correct reporting pathway for suspected abuse

EVALUATION & DOCUMENTATION

- Privacy return demonstration (satisfactory required)
- Oral questioning on rights and reporting duties
- Hours documented on Part I tracking form

INSTRUCTOR NOTES

Privacy and dignity steps are embedded in every state-exam skill. From this session forward, deduct checklist points in all labs when privacy steps are skipped.

Lesson Plan I-F: Legal & Ethical Behavior and Scope of Practice

Curriculum Part	Part I-F
Setting	GAHCTC Skills Laboratory
Allocated Hours	4 hours (pre-clinical)
Textbook Reference	Hartman 6e, Ch. 1, pp. 4–20 (nursing assistant role and scope, care team and chain of command, policies/procedures/professionalism, legal and ethical behavior, Residents' Rights, medical record, MDS, incident reports); Ch. 10, p. 254 (certification and the state registry)
GA NATP / OBRA Note	GA NATP Mod 01 · Mod 06 · Legal/Ethical Federal 16-hr rule
Pre-Clinical Requirement	YES — required before resident contact

LEARNING OBJECTIVES

Upon completion, the student will be able to:

1. Define the CNA scope of practice and identify tasks a CNA must refuse and report.
2. Distinguish abuse, neglect, negligence, assault, battery, false imprisonment, and involuntary seclusion.
3. Demonstrate ethically correct responses to gifts, tips, personal requests, and boundary challenges.
4. Explain informed consent and the resident's right to refuse care, including the CNA's response.
5. Describe accurate, legal documentation principles (objective, timely, never pre-charted, error-correction rules).

MATERIALS & EQUIPMENT

- Scope-of-practice sorting cards ('CNA task' vs. 'not a CNA task')
- Ethical dilemma scenario deck; sample documentation/flow sheets
- GA DCH nurse aide registry information sheet

LESSON SEQUENCE & TIME ALLOCATION

Time	Activity	Method
0:00–0:40	Scope of practice card sort in teams; debrief on refusing delegated tasks outside scope	Active learning
0:40–1:30	Legal terms applied: match real-world scenarios to abuse/neglect/negligence categories; consequences including registry findings	Case analysis
1:30–2:20	Ethics rotation: the gift, the secret, the shortcut, the social media post — small groups present responses	Role play
2:20–3:10	Right to refuse: demonstration of respectful response to refusal, re-approach, and reporting to the nurse; documentation practice on flow sheet	Demo + practice
3:10–4:00	Comprehensive Part I review game (Jeopardy-style) covering I-A through I-F; Part I hour verification and sign-off	Review + evaluation

SKILLS CHECKLIST — RETURN DEMONSTRATION REQUIRED

Each skill is demonstrated by the instructor, practiced under supervision, and validated by return demonstration on the GAHCTC skills checklist. Students must achieve 100% on critical steps before clinical placement.

- Correctly sorts in-scope vs. out-of-scope tasks and verbalizes refusal/reporting language
- Responds appropriately to a resident's refusal of care
- Completes a sample flow-sheet entry that is objective, accurate, and properly corrected

EVALUATION & DOCUMENTATION

- Team card-sort accuracy and scenario rubric
- Documentation sample graded against legal charting criteria
- Part I completion form signed by Instructor of Record (federal 16-hr requirement verified: 22 hrs completed)

INSTRUCTOR NOTES

Close Part I by verifying every student's pre-clinical hours and competencies. No student proceeds to Part II labs or clinical without a signed Part I completion form.

PART II — Basic Nursing Skills (3 Lab Hours)

Lesson Plan II-1: Vital Signs, Height/Weight, Environment, Observation & Reporting, End-of-Life Care

Curriculum Part	Part II A–F
Setting	GAHCTC Skills Laboratory
Allocated Hours	3 hours
Textbook Reference	Hartman 6e, Ch. 7, pp. 175–216 (admission/transfer/discharge, vital signs, height and weight, restraints, I&O, catheter/oxygen/IV care, resident's unit, bedmaking, dressings); Ch. 3, pp. 75–82 (care of residents who are dying, hospice goals); Ch. 4, pp. 83–114 (body systems and related conditions); Ch. 6, p. 128 (personal care of residents)
GA NATP / OBRA Note	GA NATP Mod 08 (Vital Signs) · Mod 07 (Observation) · Mod 15 (End of Life)
Pre-Clinical Requirement	No — Part I must be complete

LEARNING OBJECTIVES

Upon completion, the student will be able to:

1. Measure and record temperature, pulse, respirations, and blood pressure within instructor-verified accuracy parameters.
2. Measure and record height and weight using standing and chair scales.
3. Identify normal adult vital sign ranges and values that must be reported immediately.
4. Maintain a safe, clean, comfortable resident unit (bed position, call light, clutter, water within reach).
5. Recognize and report abnormal changes in body function, including the observation list in the state curriculum (breathing changes, skin changes, urinary changes, behavior changes, intake changes).
6. Recognize and report resident pain using observation and resident report.
7. Describe care of the resident when death is imminent and demonstrate the principles of post-mortem care.

MATERIALS & EQUIPMENT

- Manual and electronic BP cuffs (multiple sizes), stethoscopes, alcohol wipes
- Digital and electronic thermometers with probe covers; watch with second hand
- Standing scale and wheelchair/chair scale; vital signs recording flow sheets
- Resident unit setup (bed, over-bed table, call light); post-mortem care supply demonstration kit
- Hartman skills videos: TPR, Blood Pressure, Height/Weight

LESSON SEQUENCE & TIME ALLOCATION

Time	Activity	Method
0:00–0:50	Vital signs circuit: instructor demo, then stations for temperature, radial pulse, respirations, and two-step manual BP; students record on flow sheets; instructor verifies accuracy against control readings	Demo + return demo
0:50–1:15	Height and weight: standing and chair scale technique, balancing, recording; when weight change must be reported	Demo + return demo

Time	Activity	Method
1:15–1:45	Observation & reporting lab: 'what do you see?' photo/scenario cards drawn from the state observation list — students verbalize the finding and the report to the nurse	Active learning
1:45–2:10	Resident environment: complete a unit safety/comfort check; pain recognition cues in verbal and nonverbal residents	Practice
2:10–2:40	End-of-life: signs of approaching death, comfort measures, family support, dignity; post-mortem care demonstration and guided walk-through	Demo + discussion
2:40–3:00	Checklist validation and documentation; identify students needing extra vital-signs practice before clinical	Evaluation

SKILLS CHECKLIST — RETURN DEMONSTRATION REQUIRED

Each skill is demonstrated by the instructor, practiced under supervision, and validated by return demonstration on the GAHCTC skills checklist. Students must achieve 100% on critical steps before clinical placement.

- Counts and records radial pulse and respirations (± 4 of instructor's reading)
- Measures temperature with electronic thermometer
- Measures manual blood pressure (± 8 mmHg systolic/diastolic of instructor's reading)
- Measures and records weight (± 2 lbs) and height
- Verbalizes abnormal findings requiring immediate report
- Verbalizes/walks through post-mortem care with dignity measures

EVALUATION & DOCUMENTATION

- Accuracy-verified return demonstrations recorded on skills checklists
- Observation scenario responses graded for objectivity and correct reporting
- Open lab hours offered for additional BP practice before clinical

INSTRUCTOR NOTES

Blood pressure is the highest-failure skill on the state exam. Schedule struggling students for open-lab remediation during Week 3.

PART III — Personal Care Skills (4 Lab Hours)

Lesson Plan III-1: Bathing, Grooming, Dressing, Toileting, Feeding, Skin Care, Transfers & Positioning

Curriculum Part	Part III A–H
Setting	GAHCTC Skills Laboratory
Allocated Hours	4 hours
Textbook Reference	Hartman 6e, Ch. 6, pp. 128–174 (personal care, skin care and pressure injuries, bathing, grooming, dressing, oral hygiene, elimination, positioning and moving); Ch. 8, pp. 217–234 (nutrients and MyPlate, food preferences, special diets, fluid balance, weight loss, appetite, assisting with eating, swallowing problems, special needs)
GA NATP / OBRA Note	GA NATP Mod 09 (Personal Care) · Mod 13 (Skin/Wound) · Mod 10 (Nutrition assist) — 100% on checklists before clinical
Pre-Clinical Requirement	No — Part I must be complete

LEARNING OBJECTIVES

Upon completion, the student will be able to:

1. Demonstrate a partial bed bath and perineal care with privacy, safety, and skin observation for pressure injuries and skin tears.
2. Demonstrate grooming skills: oral care (including care for an unconscious resident and dentures), hair care, nail/foot care within CNA scope, and shaving safety principles.
3. Demonstrate dressing a resident with a weak/affected side (affected side in first, out last).
4. Demonstrate toileting assistance: bedpan, urinal, and bedside commode with privacy and skin checks.
5. Demonstrate feeding a dependent resident and assisting with hydration using safe feeding techniques (upright position, small bites, pacing, aspiration precautions).
6. Demonstrate positioning (supine, lateral, Fowler's) and turning with pressure-injury prevention, and safe transfers bed-to-wheelchair using a gait belt.

MATERIALS & EQUIPMENT

- Beds with manikins; bath basins, washcloths, towels, bath blankets, gloves
- Oral care kits, denture cups, grooming supplies, adaptive clothing items
- Bedpans, urinals, bedside commode; gait belts; wheelchairs; positioning pillows/wedges
- Meal trays with varied textures (simulation), adaptive utensils, clothing protectors
- Pressure-injury staging photo cards; Hartman skills videos for each personal care skill

LESSON SEQUENCE & TIME ALLOCATION

Time	Activity	Method
0:00–0:50	Bathing & skin care station: partial bed bath and perineal care demo on manikin; skin inspection for pressure injuries and skin tears at bony prominences; students practice in pairs	Demo + return demo

Time	Activity	Method
0:50–1:30	Grooming station: oral care (conscious, unconscious, dentures), hair and nail care; scope limits for foot/nail care in diabetic residents	Demo + return demo
1:30–2:05	Dressing station: dressing a resident with right-sided weakness; promoting independence with cueing (links to Part I-D)	Demo + return demo
2:05–2:40	Toileting station: bedpan placement, urinal, bedside commode transfer; privacy, hygiene, output observation	Demo + return demo
2:40–3:15	Feeding & hydration station: feeding a dependent resident; aspiration precautions; encouraging fluids; intake estimation and recording	Demo + return demo
3:15–3:50	Transfers & positioning station: gait-belt transfer bed↔wheelchair; turning and repositioning on schedule; supine/lateral/Fowler's alignment	Demo + return demo
3:50–4:00	Checklist completion and remediation plan; 100% competency gate confirmed	Evaluation

SKILLS CHECKLIST — RETURN DEMONSTRATION REQUIRED

Each skill is demonstrated by the instructor, practiced under supervision, and validated by return demonstration on the GAHCTC skills checklist. Students must achieve 100% on critical steps before clinical placement.

- Partial bed bath with perineal care (female and male technique)
- Oral care — conscious resident; oral care — unconscious resident; denture care
- Dressing resident with an affected/weak side
- Bedpan assistance; bedside commode transfer
- Feeding a dependent resident with aspiration precautions
- Gait-belt transfer bed to wheelchair; positioning supine/lateral/Fowler's; turning in bed

EVALUATION & DOCUMENTATION

- 100% on all Part III skill checklists required before clinical (per course map competency gate)
- Skin-observation callouts required during every bathing/positioning demo ('what would you report?')
- Remediation and open-lab retesting scheduled before Week 4 for any unsatisfactory skill

INSTRUCTOR NOTES

Rotate students through all six stations; keep gloves and hand-hygiene expectations live at every station (deduct for lapses, reinforcing Part I-B).

PART IV — Mental Health & Social Service Needs (1 Lab Hour)

Lesson Plan IV-1: Responding to Resident Behavior, Emotional Needs, Family, Spiritual & Cultural Care

Curriculum Part	Part IV A–G
Setting	GAHCTC Skills Laboratory
Allocated Hours	1 hour
Textbook Reference	Hartman 6e, Ch. 3, pp. 63–82 (basic human needs, holistic care, cultural differences, family roles, growth and development, developmental disabilities, mental health disorders, dying residents, hospice); Ch. 2, pp. 24–30 (barriers to communication; residents with special needs)
GA NATP / OBRA Note	GA NATP Mod 05 · Mod 14 — psychosocial needs; OBRA quality-of-life mandate
Pre-Clinical Requirement	No — Part I must be complete

LEARNING OBJECTIVES

Upon completion, the student will be able to:

1. Modify the aide's own behavior in response to resident behavior, including signs of depression and withdrawal.
2. Identify developmental tasks associated with aging and describe how they shape resident behavior.
3. Demonstrate de-escalating and supportive responses to anger, crying, withdrawal, and inappropriate behavior.
4. Demonstrate reinforcing resident dignity by honoring personal choices during care.
5. Describe how to engage a resident's family as a source of emotional support, and how to meet spiritual and cultural needs within the CNA role.

MATERIALS & EQUIPMENT

- Behavior-response scenario cards (angry resident, tearful resident, withdrawn resident, resident refusing meals)
- Cultural/spiritual preference cards (dietary, modesty, prayer, end-of-life customs)
- Depression warning-signs handout keyed to observation/reporting list

LESSON SEQUENCE & TIME ALLOCATION

Time	Activity	Method
0:00–0:10	Aging and developmental tasks recap; behavior as communication	Discussion
0:10–0:35	Behavior-response role plays in triads (resident, CNA, observer): supportive responses, when to step back, what to report — includes depression indicators	Role play
0:35–0:50	Culture and spirituality rotation: honoring preferences during care; family as partners; scope limits (never impose beliefs)	Case discussion
0:50–1:00	Debrief, reporting language for mood/behavior changes, checklist sign-off	Evaluation

SKILLS CHECKLIST — RETURN DEMONSTRATION REQUIRED

Each skill is demonstrated by the instructor, practiced under supervision, and validated by return demonstration on the GAHCTC skills checklist. Students must achieve 100% on critical steps before clinical placement.

- Demonstrates a calm, supportive response to a distressed resident scenario
- Verbalizes signs of depression requiring report to the nurse
- Honors a stated cultural or spiritual preference during a simulated care task

EVALUATION & DOCUMENTATION

- Observed role-play rubric (empathy, safety, reporting)
- Oral questioning on depression indicators and reporting pathway

INSTRUCTOR NOTES

Emphasize that behavior changes appear on the Part II observation/reporting list — mental health observations are reportable findings, not judgments.

PART V — Care of Cognitively Impaired Residents (1 Lab Hour)

Lesson Plan V-1: Dementia Care: Communication, Behavior, and Response Techniques

Curriculum Part	Part V A–E
Setting	GAHCTC Skills Laboratory
Allocated Hours	1 hour
Textbook Reference	Hartman 6e, Ch. 5, pp. 115–127 (confusion and delirium, dementia and Alzheimer's disease, communication strategies, ADL interventions, difficult behaviors, creative therapies); Ch. 3, pp. 63–66 (basic human needs, holistic care, independence, cultural differences)
GA NATP / OBRA Note	GA NATP Mod 06 — Alzheimer's/dementia care
Pre-Clinical Requirement	No — Part I must be complete

LEARNING OBJECTIVES

Upon completion, the student will be able to:

1. Describe techniques for addressing the unique needs and behaviors of residents with Alzheimer's disease and other dementias.
2. Demonstrate communication techniques for cognitively impaired residents (approach from front, simple one-step instructions, calm tone, validation).
3. Interpret common dementia behaviors (wandering, sundowning, agitation, catastrophic reactions, hoarding) as unmet needs.
4. Demonstrate appropriate responses: redirection, validation, distraction — never arguing, correcting, or restraining.
5. Identify environmental methods that reduce the effects of cognitive impairment (routine, cues, reduced stimulation).

MATERIALS & EQUIPMENT

- Dementia behavior scenario cards; simple-instruction task props (folding towels, sorting items)
- Communication do/don't quick-reference cards
- Optional: virtual dementia experience elements (gloves, glasses, headphones) if available

LESSON SEQUENCE & TIME ALLOCATION

Time	Activity	Method
0:00–0:10	Dementia vs. normal aging vs. delirium quick review; behavior as unmet need	Discussion
0:10–0:35	Communication practice: paired drills using one-step instructions and validation therapy responses; approach and body position	Guided practice
0:35–0:50	Behavior rotation: wandering, sundowning, refusal, catastrophic reaction — students demonstrate redirection/distraction responses	Role play + return demo
0:50–1:00	Environment and routine strategies; reporting behavior changes; sign-off	Evaluation

SKILLS CHECKLIST — RETURN DEMONSTRATION REQUIRED

Each skill is demonstrated by the instructor, practiced under supervision, and validated by return demonstration on the GAHCTC skills checklist. Students must achieve 100% on critical steps before clinical placement.

- Demonstrates correct approach and one-step communication with a cognitively impaired resident
- Demonstrates redirection/validation in response to agitation without arguing or restraint
- Verbalizes safety response to a wandering/elopement scenario

EVALUATION & DOCUMENTATION

- Role-play rubric (approach, tone, technique, safety)
- Oral questioning on restraint-free responses (links to Part VII-G)

INSTRUCTOR NOTES

Students will apply these techniques on the memory-care assignment during Clinical Day 3.

PART VI — Basic Restorative Services (2 Lab Hours)

Lesson Plan VI-1: Restorative Nursing: Self-Care Training, Ambulation, Assistive Devices, ROM, Positioning, Bowel & Bladder Training

Curriculum Part	Part VI A–H
Setting	GAHCTC Skills Laboratory
Allocated Hours	2 hours
Textbook Reference	Hartman 6e, Ch. 9, pp. 235–248 (rehabilitation and restorative care, exercise and independence, ambulation and assistive devices, body alignment, prosthetic devices, range of motion, bladder and bowel retraining); Ch. 6, pp. 162–174 (positioning and moving residents); Ch. 8, p. 233 (residents with special needs) · Procedures: ambulation p. 237; cane/walker/crutches p. 239; passive ROM p. 243
GA NATP / OBRA Note	GA NATP Mod 09 (Mobility) — restorative principles per OBRA
Pre-Clinical Requirement	No — Parts I and III must be complete

LEARNING OBJECTIVES

Upon completion, the student will be able to:

1. Explain restorative nursing principles and the CNA's role in training residents in self-care at their ability level.
2. Demonstrate ambulation assistance with and without a gait belt, including guarding technique and fall response during ambulation.
3. Demonstrate assisting residents who use assistive devices (cane, walker, wheelchair) for transferring, ambulation, eating, and dressing.
4. Perform passive range-of-motion exercises for upper and lower extremities safely (support joints, never force, stop at pain).
5. Demonstrate proper turning and positioning in bed and chair on a repositioning schedule.
6. Describe the CNA's role in bowel and bladder training programs, including scheduled toileting and accurate recording.
7. Demonstrate care and application assistance for prosthetic/orthotic devices and eyeglasses.

MATERIALS & EQUIPMENT

- Gait belts, canes (single-point/quad), walkers, wheelchairs
- Beds with manikins for ROM and positioning; positioning devices
- Adaptive eating and dressing equipment (built-up utensils, plate guards, reachers, sock aids)
- Sample bowel/bladder training record; eyeglass and prosthetic care demonstration items

LESSON SEQUENCE & TIME ALLOCATION

Time	Activity	Method
0:00–0:15	Restorative philosophy: restore, maintain, prevent decline; connecting to promoting independence (Part I-D)	Discussion
0:15–0:50	Ambulation station: gait-belt application, assisted ambulation, guarding, controlled lowering in a fall; cane/walker gait patterns	Demo + return demo

Time	Activity	Method
0:50–1:20	ROM station: passive ROM sequence — shoulder, elbow, wrist, hip, knee, ankle — on manikin/peer with joint support and pain-stop rule	Demo + return demo
1:20–1:40	Positioning & devices station: turning schedule practice in bed and chair; adaptive equipment for eating and dressing; eyeglasses and prosthetic/orthotic care	Demo + practice
1:40–1:55	Bowel & bladder training: scheduled toileting walk-through; recording on training log; dignity and praise, never scolding	Demo + discussion
1:55–2:00	Checklist sign-off and remediation scheduling	Evaluation

SKILLS CHECKLIST — RETURN DEMONSTRATION REQUIRED

Each skill is demonstrated by the instructor, practiced under supervision, and validated by return demonstration on the GAHCTC skills checklist. Students must achieve 100% on critical steps before clinical placement.

- Applies gait belt and assists with ambulation safely
- Performs passive ROM on one upper and one lower extremity
- Repositions resident in bed and chair with alignment and pressure relief
- Demonstrates set-up of adaptive equipment for eating or dressing
- Verbalizes CNA role in a bowel/bladder training program

EVALUATION & DOCUMENTATION

- Return demonstration checklists (ambulation and ROM satisfactory required)
- Scenario response for fall-during-ambulation (controlled lowering, stay, call, report)

INSTRUCTOR NOTES

Ambulation with gait belt and passive ROM are common state-exam skills — run timed practice in the Georgia testing format.

PART VII — Residents' Rights in Practice (1 Lab Hour)

Lesson Plan VII-1: Demonstrating Behavior That Maintains Residents' Rights

Curriculum Part	Part VII A–G
Setting	GAHCTC Skills Laboratory
Allocated Hours	1 hour
Textbook Reference	Hartman 6e, Ch. 1, pp. 10–20 (legal and ethical behavior, Residents' Rights, medical record, MDS, incident reports); Ch. 3, pp. 66–68 (cultural differences, need for activity, family roles); Ch. 7, pp. 195–197 (restraints), pp. 208–209 (resident's unit and related care)
GA NATP / OBRA Note	GA NATP Mod 05 — behavioral demonstration requirement (builds on Part I-E)
Pre-Clinical Requirement	No — Part I must be complete; capstone before clinical

LEARNING OBJECTIVES

Upon completion, the student will be able to:

1. Demonstrate providing privacy and maintaining confidentiality during simulated care.
2. Demonstrate promoting the resident's right to make personal choices to accommodate their needs.
3. Describe how to assist residents in resolving grievances and disputes, including the role of the ombudsman.
4. Demonstrate assisting a resident to get to and participate in resident/family group activities.
5. Demonstrate maintaining care and security of residents' personal possessions.
6. Demonstrate care free from abuse, mistreatment, and neglect, and the reporting pathway for any observed violation.
7. Identify restraint alternatives consistent with current professional standards.

MATERIALS & EQUIPMENT

- Integrated rights scenario stations combining care tasks with rights challenges
- Personal possessions inventory form; grievance pathway poster; restraint-alternatives cards

LESSON SEQUENCE & TIME ALLOCATION

Time	Activity	Method
0:00–0:10	Rights-in-action briefing: this hour is behavioral demonstration, not lecture	Instructor-led
0:10–0:45	Integrated stations: (1) care task with privacy + choice offered; (2) resident voices a grievance — student responds and routes correctly; (3) roommate conflict over possessions — inventory and security; (4) observed rough handling by 'another aide' — student demonstrates reporting	Simulation circuit + return demo
0:45–0:55	Restraint alternatives rapid round: student names an alternative for each scenario (wandering, fall risk, agitation)	Active learning
0:55–1:00	Final pre-clinical readiness verification and sign-off	Evaluation

SKILLS CHECKLIST — RETURN DEMONSTRATION REQUIRED

Each skill is demonstrated by the instructor, practiced under supervision, and validated by return demonstration on the GAHCTC skills checklist. Students must achieve 100% on critical steps before clinical placement.

- Integrates privacy, choice, and dignity into a complete simulated care task
- Responds to and correctly routes a resident grievance (nurse, ombudsman awareness)
- Demonstrates immediate reporting of observed mistreatment
- Names appropriate restraint alternatives for common scenarios

EVALUATION & DOCUMENTATION

- Integrated-station rubric — this serves as the lab capstone for rights competency
- Pre-clinical readiness checklist finalized: Parts I–VII lab competencies verified before Week 4

INSTRUCTOR NOTES

This capstone hour verifies that rights behaviors are habitual before students touch a real resident. Any deficiency triggers remediation before clinical.

CLINICAL PRACTICE — 24 Hours (Mon–Wed, Week 4, 7:00 a.m.–3:00 p.m.)

All clinical experiences occur at the affiliated long-term care facility under the direct supervision of the RN Instructor of Record. Students wear the GAHCTC uniform and ID, arrive by 6:45 a.m. for pre-conference, and may perform only skills validated in lab. Each day includes pre-conference (7:00–7:30), supervised resident care, lunch (30 min), and post-conference (2:30–3:00).

Lesson Plan C-1: Clinical Day 1 — Orientation, Infection Control & Personal Care Fundamentals

Curriculum Part	Applies Parts I, III, VII
Setting	Affiliated LTC facility (clinical)
Allocated Hours	8 hours (7:00 a.m.–3:00 p.m.)
Textbook Reference	Hartman 6e, Ch. 2, pp. 21–62 and Ch. 6, pp. 128–174 (applied)
GA NATP / OBRA Note	Georgia DCH clinical supervision requirements; instructor validates each skill before independent-with-supervision practice
Pre-Clinical Requirement	Parts I–VII lab competencies verified and signed

LEARNING OBJECTIVES

Upon completion, the student will be able to:

1. Navigate the facility safely: fire exits, emergency codes, supply and linen rooms, and chain of command on the assigned unit.
2. Demonstrate hand hygiene and standard precautions consistently throughout resident care (revalidated on site).
3. Provide morning care (partial bath, oral care, grooming, dressing) for 1–2 assigned residents with privacy and dignity.
4. Make an occupied and unoccupied bed to facility standard.
5. Apply residents' rights behaviors in every interaction and report observations objectively to the nurse and instructor.

MATERIALS & EQUIPMENT

- Clinical skills validation checklists (per student); assignment sheets (1–2 residents per student, Day 1)
- Facility orientation packet; student clinical evaluation forms; daily observation journals

LESSON SEQUENCE & TIME ALLOCATION

Time	Activity	Method
7:00–7:30	Pre-conference: facility orientation, unit tour, resident assignments, safety and privacy expectations; on-site hand hygiene revalidation	Instructor-led
7:30–8:00	Shadow report: students observe nurse–aide handoff; identify their residents' care needs from assignment sheet	Observation
8:00–11:30	Supervised a.m. care: bathing, oral care, grooming, dressing, bedmaking, toileting assistance for assigned residents; instructor directly supervises and validates each first-time skill	Direct supervised practice

Time	Activity	Method
11:30–12:00	Student lunch (staggered to maintain resident coverage)	—
12:00–1:00	Meal assistance: dining room set-up, feeding dependent residents with aspiration precautions, intake recording	Direct supervised practice
1:00–2:30	Afternoon rounds: repositioning schedule, hydration passes, call lights, unit environment checks; begin clinical journal entry	Supervised practice
2:30–3:00	Post-conference: skill validations reviewed, observations debriefed, Day 2 assignments issued	Debrief

SKILLS CHECKLIST — RETURN DEMONSTRATION REQUIRED

Each skill is demonstrated by the instructor, practiced under supervision, and validated by return demonstration on the GAHCTC skills checklist. Students must achieve 100% on critical steps before clinical placement.

- On-site revalidation: hand hygiene; gloving
- Partial bath / perineal care (first on-resident validation)
- Oral care; dressing; occupied and unoccupied bedmaking
- Feeding a dependent resident; intake recording

EVALUATION & DOCUMENTATION

- Clinical validation checklist signed per skill by RN instructor
- Daily clinical evaluation form (professionalism, safety, communication, rights)
- Clinical journal entry: one observation reported and what was done with it

INSTRUCTOR NOTES

Day 1 assignments favor cooperative, lower-acuity residents. Any unsafe practice stops immediately for on-the-spot coaching and is documented.

Lesson Plan C-2: Clinical Day 2 — Vital Signs, Mobility & Restorative Care

Curriculum Part	Applies Parts II, VI
Setting	Affiliated LTC facility (clinical)
Allocated Hours	8 hours (7:00 a.m.–3:00 p.m.)
Textbook Reference	Hartman 6e, Ch. 7, pp. 175–216 and Ch. 9, pp. 235–248 (applied); Ch. 8, pp. 217–234
GA NATP / OBRA Note	Georgia DCH clinical supervision requirements
Pre-Clinical Requirement	Clinical Day 1 satisfactorily completed

LEARNING OBJECTIVES

Upon completion, the student will be able to:

1. Measure and record vital signs, height, and weight on assigned residents with instructor verification.
2. Recognize and report abnormal findings and resident pain using the Part II observation list.
3. Perform safe gait-belt transfers and assisted ambulation with assigned residents.
4. Perform passive ROM and scheduled repositioning as care-planned.
5. Support restorative programs: self-care prompting, adaptive equipment at meals, scheduled toileting with accurate recording.

MATERIALS & EQUIPMENT

- BP cuffs, stethoscopes, thermometers, facility scales; gait belts
- Vital signs flow sheets; restorative/toileting logs; validation checklists

LESSON SEQUENCE & TIME ALLOCATION

Time	Activity	Method
7:00–7:30	Pre-conference: expanded assignments (2–3 residents); review of each resident's mobility status, transfer orders, and vital-sign parameters	Instructor-led
7:30–9:00	Vital signs rounds: TPR, BP, and weights on assigned residents; instructor double-checks readings; abnormal values reported to nurse	Direct supervised practice
9:00–11:30	A.M. care with mobility focus: gait-belt transfers, assisted ambulation to dining/activities, repositioning schedule, passive ROM per care plan	Direct supervised practice
11:30–12:00	Student lunch (staggered)	—
12:00–1:00	Restorative dining: adaptive equipment set-up, self-feeding prompting, feeding assistance where needed; intake/output recording	Supervised practice
1:00–2:30	Afternoon care: scheduled toileting program participation, hydration, ambulation passes, pain observation rounds; journal entry	Supervised practice
2:30–3:00	Post-conference: vital-sign accuracy review, mobility debrief, Day 3 comprehensive assignment issued	Debrief

SKILLS CHECKLIST — RETURN DEMONSTRATION REQUIRED

Each skill is demonstrated by the instructor, practiced under supervision, and validated by return demonstration on the GAHCTC skills checklist. Students must achieve 100% on critical steps before clinical placement.

- Vital signs on residents (instructor-verified accuracy)
- Height/weight on facility scales
- Gait-belt transfer and assisted ambulation (on-resident validation)
- Passive ROM; repositioning per schedule
- Restorative feeding set-up and prompting

EVALUATION & DOCUMENTATION

- Clinical validation checklist per skill; vital signs accuracy log
- Daily clinical evaluation form; journal entry on a reported abnormal finding or pain observation

INSTRUCTOR NOTES

Pair each student's resident load to include at least one resident with a transfer/ambulation order and one on a restorative or toileting program.

Lesson Plan C-3: Clinical Day 3 — Comprehensive Care, Dementia Unit & Final Competency Validation

Curriculum Part	Applies Parts I–VII (integration)
Setting	Affiliated LTC facility (clinical)
Allocated Hours	8 hours (7:00 a.m.–3:00 p.m.)
Textbook Reference	Hartman 6e, Ch. 3, pp. 63–82; Ch. 5, pp. 115–127; all applied chapters (Ch. 1–10)
GA NATP / OBRA Note	Georgia DCH clinical hour completion (24:00) verified this day
Pre-Clinical Requirement	Clinical Days 1–2 satisfactorily completed

LEARNING OBJECTIVES

Upon completion, the student will be able to:

1. Manage a comprehensive assignment (3–4 residents) integrating personal care, vital signs, mobility, nutrition, and rights behaviors with decreasing prompting.
2. Apply dementia-care communication and behavior-response techniques during a memory-care rotation.
3. Demonstrate psychosocial support: responding to resident emotions, engaging family appropriately, honoring cultural/spiritual preferences.
4. Report and record observations across the full state observation list, including behavior and intake changes.
5. Complete all outstanding clinical skill validations for program completion.

MATERIALS & EQUIPMENT

- Comprehensive assignment sheets; memory-care rotation schedule
- Final clinical competency summary form; program clinical hour log (24-hr verification)

LESSON SEQUENCE & TIME ALLOCATION

Time	Activity	Method
7:00–7:30	Pre-conference: comprehensive assignments; memory-care rotation groups; review of outstanding validations per student	Instructor-led
7:30–11:30	Comprehensive care block: students organize and deliver full a.m. care for 3–4 residents (care, vitals as assigned, transfers, repositioning), with instructor validating remaining skills; half-group rotates through memory-care unit applying Part V techniques	Supervised practice + validation
11:30–12:00	Student lunch (staggered)	—
12:00–1:00	Dining assistance including memory-care dining; hydration and intake recording	Supervised practice
1:00–2:00	Second memory-care rotation group; remaining students complete afternoon rounds and any final validations	Supervised practice + validation
2:00–2:30	Resident farewells; unit restocked; documentation completed and co-signed	Practice

Time	Activity	Method
2:30–3:00	Final post-conference: competency summary review, clinical hour log signed (24:00 verified), self-evaluation, transition to Program Conclusion & Certification Preparation (5:30 hrs)	Evaluation & debrief

SKILLS CHECKLIST — RETURN DEMONSTRATION REQUIRED

Each skill is demonstrated by the instructor, practiced under supervision, and validated by return demonstration on the GAHCTC skills checklist. Students must achieve 100% on critical steps before clinical placement.

- Comprehensive multi-resident assignment completed safely and on time
- Dementia communication and redirection applied with a memory-care resident
- All remaining program skill validations completed

EVALUATION & DOCUMENTATION

- Final Clinical Competency Summary signed by RN Instructor of Record
- 24 clinical hours verified on program hour log (Mon–Wed complete)
- Summative clinical evaluation (safety, skills, communication, rights, professionalism) — satisfactory required for program completion and state exam eligibility

INSTRUCTOR NOTES

Students with any outstanding validation receive a documented remediation plan before the certification-preparation block. Celebrate completion — this cohort moves to state exam readiness.

Appendix A — Hartman 6e Chapter Map to the Federal & State Core Curriculum

This map aligns Hartman’s Nursing Assistant Care: The Basics, 6th Edition (Fuzy/Hartman Publishing, ISBN 978-1-60425-141-8) to each lesson plan in this packet. Page numbers reflect the chapter learning objectives as printed in the textbook contents. Instructors should assign the online TalentLMS module and the corresponding reading before the matching lab session.

Hartman 6e Chapter	Learning Objectives & Page Locations	Curriculum Part / Lesson Plan
Ch. 1 — The Nursing Assistant in Long-Term Care (pp. 1–20)	Long-term care vs. other settings 1; typical LTC facility 2; Medicare and Medicaid 3; nursing assistant’s role 4; care team and chain of command 4; policies, procedures, professionalism 8; legal and ethical behavior and Residents’ Rights 10; legal aspects of the medical record 18; Minimum Data Set (MDS) 20; incident reports 20	I-A · I-D · I-E · I-F · VII-1
Ch. 2 — Foundations of Resident Care (pp. 21–62)	Verbal and written communication 21; barriers to communication 24; communicating with residents with special needs 26; safety and nonmedical emergencies 31; recognizing and responding to medical emergencies 37; infection prevention and control 46	I-A · I-B · I-C · IV-1 · C-1
Ch. 3 — Understanding Residents (pp. 63–82)	Basic human needs 63; holistic care 65; promoting independence and self-care 65; cultural differences 66; need for activity 68; family roles 68; human growth and development 69; developmental disabilities 72; mental health disorders 73; residents who are dying 75; goals of hospice 80	I-D · I-E · II-1 · IV-1 · V-1 · C-3
Ch. 4 — Body Systems and Related Conditions (pp. 83–114)	Integumentary 83; musculoskeletal 84; nervous 88; circulatory 94; respiratory 98; urinary 100; gastrointestinal 102; endocrine 106; reproductive 109; immune and lymphatic 110	II-1 (observation & reporting) · III-1 · VI-1
Ch. 5 — Confusion, Dementia, and Alzheimer’s Disease (pp. 115–127)	Confusion and delirium 115; dementia and Alzheimer’s disease 116; communication strategies 118; interventions for ADL problems 120; interventions for difficult behaviors 122; creative therapies 126	V-1 · C-3
Ch. 6 — Personal Care Skills (pp. 128–174)	Personal care of residents 128; skin care and pressure injury prevention 129; bathing 133; grooming 142; dressing 148; oral hygiene 152; elimination 157; safe positioning and moving 162	III-1 · VI-1 · C-1
Ch. 7 — Basic Nursing Skills (pp. 175–216)	Admission, transfer, discharge 175; vital signs 179; weight and height 193; restraints and restraint-free environment 195; fluid balance and I&O 197; catheters, oxygen therapy, IV therapy 203; resident’s unit 208; sleep and bedmaking 209; dressings and bandages 214	II-1 · VII-1 · C-2
Ch. 8 — Nutrition and Hydration (pp. 217–234)	Six basic nutrients and MyPlate 217; factors influencing food preferences 221; special diets 221;	III-1 · VI-1 · C-2

Hartman 6e Chapter	Learning Objectives & Page Locations	Curriculum Part / Lesson Plan
	maintaining fluid balance 224; unintended weight loss 226; promoting appetite 227; assisting with eating 228; swallowing problems 230; residents with special needs 233	
Ch. 9 — Rehabilitation and Restorative Care (pp. 235–248)	Rehabilitation and restorative care 235; promoting independence and exercise 236; ambulation and assistive devices 237; proper body alignment 240; prosthetic devices 241; range of motion exercises 242; bladder and bowel retraining 247	VI-1 · C-2
Ch. 10 — Caring for Yourself (pp. 249–257)	Finding a job 249; job description and time management 251; managing and resolving conflict 252; employee evaluations and feedback 253; certification and the state registry 254; continuing education 254; managing stress 255	I-A (stress) · I-F (registry/scope) · Program Conclusion & Certification Preparation
Back matter	Abbreviations 258; Glossary 259; Index 270	Student reference throughout the program

Appendix B — Hartman 6e Procedure Page Index (Lab & Clinical Skills)

Every procedure listed in the Hartman 6e procedures index is cross-referenced below to the lesson plan in which it is demonstrated and validated. Instructors demonstrate the procedure as written, students practice under supervision, and return demonstration is recorded on the GAHCTC skills checklist. Procedures beyond the tested state skill list are taught for scope awareness and clinical readiness.

Hartman 6e Procedure	Page	Lesson Plan
Washing hands (hand hygiene)	50	I-B Infection Control
Putting on (donning) and removing (doffing) gown	51	I-B Infection Control
Putting on (donning) mask and goggles	52	I-B Infection Control
Putting on (donning) gloves	53	I-B Infection Control
Removing (doffing) gloves	54	I-B Infection Control
Performing abdominal thrusts for the conscious person	38	I-C Safety & Emergency
Responding to shock	39	I-C Safety & Emergency
Responding to a myocardial infarction	40	I-C Safety & Emergency
Controlling bleeding	41	I-C Safety & Emergency
Treating burns	41	I-C Safety & Emergency
Responding to fainting	42	I-C Safety & Emergency
Responding to seizures	43	I-C Safety & Emergency
Responding to vomiting	45	I-C Safety & Emergency
Admitting a resident	176	II-1 Basic Nursing Skills
Transferring a resident	177	II-1 Basic Nursing Skills
Discharging a resident	178	II-1 Basic Nursing Skills
Measuring and recording an oral temperature	181	II-1 Basic Nursing Skills
Measuring and recording a rectal temperature	183	II-1 Basic Nursing Skills
Measuring and recording a tympanic temperature	184	II-1 Basic Nursing Skills
Measuring and recording an axillary temperature	185	II-1 Basic Nursing Skills
Counting and recording radial pulse and respirations	187	II-1 Basic Nursing Skills
Measuring and recording blood pressure manually	189	II-1 Basic Nursing Skills
Measuring and recording blood pressure electronically	190	II-1 Basic Nursing Skills
Measuring and recording weight of an ambulatory resident	193	II-1 Basic Nursing Skills
Measuring and recording height of an ambulatory resident	194	II-1 Basic Nursing Skills
Measuring and recording urinary output	198	II-1 Basic Nursing Skills
Collecting a routine urine specimen	200	II-1 Basic Nursing Skills
Collecting a clean-catch (midstream) urine specimen	201	II-1 Basic Nursing Skills
Collecting a stool specimen	202	II-1 Basic Nursing Skills
Providing catheter care	204	II-1 Basic Nursing Skills

Hartman 6e Procedure	Page	Lesson Plan
Emptying the catheter drainage bag	205	II-1 Basic Nursing Skills
Making an occupied bed	210	II-1 Basic Nursing Skills
Making an unoccupied bed	213	II-1 Basic Nursing Skills
Changing a dry dressing using nonsterile technique	214	II-1 Basic Nursing Skills
Giving a complete bed bath	133	III-1 Personal Care Skills
Giving a back rub	137	III-1 Personal Care Skills
Shampooing hair in bed	139	III-1 Personal Care Skills
Giving a shower or tub bath	140	III-1 Personal Care Skills
Providing fingernail care	143	III-1 Personal Care Skills
Providing foot care	144	III-1 Personal Care Skills
Combing or brushing hair	145	III-1 Personal Care Skills
Shaving a resident	146	III-1 Personal Care Skills
Dressing a resident	149	III-1 Personal Care Skills
Applying knee-high antiembolic stockings	151	III-1 Personal Care Skills
Providing oral care	152	III-1 Personal Care Skills
Providing oral care for an unconscious resident	153	III-1 Personal Care Skills
Flossing teeth	154	III-1 Personal Care Skills
Cleaning and storing dentures	156	III-1 Personal Care Skills
Assisting a resident with the use of a bedpan	157	III-1 Personal Care Skills
Assisting a male resident with a urinal	159	III-1 Personal Care Skills
Assisting a resident to use a portable commode or toilet	161	III-1 Personal Care Skills
Caring for an ostomy	105	III-1 Personal Care Skills
Moving a resident up in bed	163	III-1 Personal Care Skills
Moving a resident to the side of the bed	164	III-1 Personal Care Skills
Positioning a resident on his side	164	III-1 Personal Care Skills
Logrolling a resident	166	III-1 Personal Care Skills
Assisting a resident to sit up on the side of the bed: dangling	167	III-1 Personal Care Skills
Transferring a resident from a bed to a wheelchair	170	III-1 Personal Care Skills
Transferring a resident using a mechanical lift	173	III-1 Personal Care Skills
Serving fresh water	225	III-1 Personal Care Skills
Assisting a resident with eating	229	III-1 Personal Care Skills
Assisting a resident to ambulate	237	VI-1 Restorative Services
Assisting with ambulation for a resident using a cane, walker, or crutches	239	VI-1 Restorative Services
Assisting with passive range of motion exercises	243	VI-1 Restorative Services

Instructor note: procedures marked for 100% competency gates (hand hygiene, PPE, transfers, feeding, perineal care) must be validated in lab before Week 4 clinical, then re-validated on Clinical Day 1 before care assignments.

Appendix C — Documentation & Compliance Checklist

Maintain the following in each student file for GA DCH program review:

- Part I pre-clinical hour log (22 hrs) with Instructor of Record signature — verifies the federal 16-hour minimum before resident contact
- Skills lab return-demonstration checklists for Parts I–VII (100% competency gates noted for infection control and personal care skills)
- Clinical validation checklists and daily clinical evaluation forms (Days 1–3)
- Clinical hour log totaling 24:00 hours
- Final Clinical Competency Summary and program completion record (98 total program hours: 40 online, 34 lab, 24 clinical)
- Remediation documentation, where applicable, with retest outcomes

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